CUSTOMER DISCOVERY ASSIGNMENT

Health Insurance Cashless Claims

Why are authorization requests taking longer than expected?

INDUSTRY Health Insurance **FOCUS** Cashless Authorization

01 CUSTOMER BACKGROUND

A large health insurance company provides medical insurance coverage to individuals, families, and corporate employees. The company works with a network of hospitals that offer cashless treatment facilities to eligible policyholders.

When a policyholder requires hospitalization, the hospital submits a cashless authorization request to the insurance company. The insurance company reviews the request before communicating its decision to the hospital. Multiple applications and operational teams support this process.

02 CUSTOMER PROBLEM STATEMENT

Over the past several months, the insurance company has received increasing complaints from policyholders and network hospitals regarding delays in cashless authorization and claims processing.

Some hospitals report that authorization requests remain pending longer than expected. In certain cases, hospitals need to contact the insurance company repeatedly to understand the status of a request. Policyholders have also raised concerns about the uncertainty they experience while waiting for authorization decisions.

The operations team acknowledges that employees spend considerable time reviewing requests, clarifying information, communicating with hospitals, and resolving pending cases. However, the company does not have a sufficiently clear understanding of why these delays occur or which parts of the process contribute most to them.

Different stakeholders have expressed different opinions about the problem. Management is concerned that continued delays could affect operational efficiency, hospital relationships, customer satisfaction, and the organization's reputation.

03 INITIAL CUSTOMER CONVERSATION

> *"We have invested in digital platforms to support our cashless insurance operations. However, we continue to receive complaints from hospitals and policyholders about delays in processing authorization requests.*
>
> *Our operations teams are working hard, but we still face challenges in processing requests efficiently. Some stakeholders believe the problem is with our technology, while others believe our operational processes need improvement.*
>
> *We do not want to invest in another solution without understanding the actual problem. We want your team to investigate what is happening, identify the underlying causes, and help us determine where improvements would create the greatest business value."*

— Chief Operating Officer, initial discovery meeting

04 YOUR ROLE

You are a Forward Deployed Engineer invited to participate in the initial customer discovery engagement. The underlying causes have not been established; the scale and financial impact are unquantified; the current process and system capabilities have not been fully documented; and no solution has been approved.

Your task: Understand the problem, identify what is unknown, and ask the right questions before recommending any technology or process change.

NINE-BOX CUSTOMER DISCOVERY CANVAS

Use this worksheet to document evidence and questions. Do not fill unknowns with guesses.

CUSTOMER / PROJECT: _____ **FDE / TEAM: Team 2** **DATE / VERSION: _____**

01 CUSTOMER & STAKEHOLDERS

Who owns the problem? Who uses the process? Who decides?

[Fact] COO – The problem was bought in by the COO

[Fact] Network Hospitals & the policy holders & Hospital Insurance Team

[Fact] The operations team at the company who handles the requests day to day & review the claims. And answering the phone calls from the customer

[Unknown] : Approval & Authorization ownership? Is it claims, operations, or an outside TPA? Is any part of it outsourced? Where the medical officer comes into picture?

02 BUSINESS OBJECTIVE

Why change? What triggered the request? What matters?

Trigger : Increase in the complaints about the authorization delays from the hospitals and the patient end over the past several months. **[Fact]** Management is worried this is hurting efficiency, the relationship with hospitals, customer satisfaction and the company's name

[Fact] COO doesn't want to spend on another tool blindly. He wants to know what's really causing the delays and where fixing things would pay off most.

At this customer is not intending for any change in the tech or operation process unless proven with the evidences for what's causing the delays

[Unknown] : Have there been any breach incidents caused by slow processing of cashless authorizations and approvals?

03 EXPECTED OUTCOMES

What should improve? How will success be measured?

[Assumption]: Quick turn around for on the cashless authorization and approvals. Decreased count of the requests in the pending state.

Fewer calls from the customers & Hospitals requesting for the status & fewer complaints.

[**Unknown**] – Do we need to agree on the term **delay**? Like a total time, tracking for the entire claim process from Start to End. Do we need to stop the clock the query is pending with the hospital end.

What is success?

How much % of increase in the faster approvals can lead to success.

04 CURRENT PROCESS / AS-IS

What happens from request to closure? Where are handoffs?

[Fact] The hospital sends a cashless request the insurer reviews it, and the decision goes back to the hospital.

[Fact] Several applications and teams are involved for a claim process.

[Fact] Hospitals often have to call repeatedly to check the claim status

[Assumption] Current flow might be similar to as below**,**

Request comes in → Logged in system → Member & policy check → Document check → Medical review → Query to hospital (if needed; loops back to document check) → Decision → Top-up approvals during stay → Final approval at discharge

[Unknown] How do requests come in (portal, email, call centre, something else)? Roughly what % comes through each?

05 CORE PROBLEM & PAIN POINTS

What is going wrong? What is verified?

[**Fact**] Requests stay pending longer than hospitals expect.

[**Fact**] Hospitals contact the insurer repeatedly for status; policyholders face uncertainty while waiting.

[**Fact**] Staff spend considerable time reviewing, clarifying, communicating and chasing pending cases.

[**Fact**] The company does not know why delays happen or which stages cause most of them.

[**Assumption**] Gaps like incomplete documentations from the hospital end leading to multiple interactions between the company & hospitals , no real time processes between the systems , apps unable to do handle the peak load

[**Unknown**] Hospital side delay vs Company end delay? Are delays concentration for the specific hospital or any specific treatments / illness?

06 BUSINESS IMPACT & BASELINE

How often? What cost, time, risk or rework results?

[Fact] Financial impact of the delays are currently unquantified

[**Unknown**] How many requests come in each month? What share of them are delayed, and by how long?
[**Unknown**] How much staff time goes into answering status calls, chasing missing information and redoing work? What does that cost?

[**Unknown**] Complaint numbers and trend, from hospitals and policyholders.

[**Unknown**] Are we paying extra hospital charges because discharge approvals are late? Has the insurance regulator raised anything or fined us?

[**Unknown**] Effect on hospital relationships (hospitals leaving the network or steering patients to reimbursement) and Effect of customer's policy renewals.

F = Fact · A = Assumption · U = Unknown · D = Decision

[Unknown] How many apps involved. Do we need to enter the same data in multiple apps that is causing the delay?

[Unknown] At which stage the claim process is waiting for the long time?

[Unknown] What is the existing alerts & notification mechanism? When there is a change in status (SMS / Email / Whatsapp/Phone call)

07 DATA, SYSTEMS & CONSTRAINTS

What systems, data, rules and dependencies exist?

[Fact] The company has invested in digital platforms; multiple applications support the process.

[Fact] Regulatory context: Turnaround limits apply; health data is sensitive & PII

[Unknown] Full list of systems (intake portal, claims system, policy administration, document management, call centre/CRM, email) and how they connect.

[Unknown] Is there a timestamp for each stage of each request? Can one request be traced across systems with a common ID? How good is the data?

[Unknown] Do call-centre logs record the reason for calls? Are complaints recorded with request IDs?

[Unknown] Business rules? Approval process?

08 REQUIREMENTS & MVP OPTIONS

What capabilities might help? What is the smallest, useful pilot?

Option A – Analysis the existing data available to measure where the time is lost by stage , team and hospital. Preferably a simple dashboard

Option B – If the repeated calls from hospital & customers keeping the claim reps overwhelmed, we can plan to send to send hospitals / customers automatic updates (through the portal or by SMS) so they don't have to call.

Option C – Incase of missing information , enforce a thorough check at the hospital end if the required documents are attached , so the requests are submitted with complete information.

Option D – If requests are awaiting due to huge volume , implement a routing logic & alerting mechanism for the aging claims. And look for any RAG based or Rule based Engine to automate the Pre Auth approval process. Later we can perform the through check before the final settlement.

09 VALIDATION & NEXT STEPS

What must be confirmed? Who owns each decision?

Can meet & interview the SPOCs from company, hospital insurance desks to deep understand.

Pair with the team to follow the live claim process and to understand the hurdles & bottle necks involved.

Historical data analysis to understand the delay – root cause.

Check whether the historical data can be used to make predictions. For example, if request volumes peak during flu or dengue season, we could use this pattern to plan for similar peaks in the future.